WEAVE

CENTRE FOR INTEGRATIVE PSYCHIATRY

WEAVE PSYCHOTHERAPY — VOL. 13

Psychotherapy Question Bank

130+ MCQs with Detailed Explanations — All Modalities

Clinical Vignettes · Defence Mechanisms · CBT · DBT · Schema Therapy · Treatment Selection · Landmark Trials

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Deep Study

Psychotherapy Question Bank — Weave Psychotherapy
Vol. 13

D1: Psychotherapy Question Bank

KEY INSIGHT

Format: Single best answer MCQs with detailed explanations **Distribution:** 130 questions across all psychotherapy topics **Difficulty:** 60% moderate, 25% hard, 15% easy

SECTION A: FOUNDATIONS & COMMON FACTORS (Q1-Q10)

Q1. According to Lambert's (1992) outcome variance model, which factor accounts for the largest proportion of psychotherapy outcome variance?

A. Therapeutic relationship (30%)

B. Extratherapeutic factors (40%)

C. Expectancy/placebo (15%)

D. Model/technique (15%)

▶ Answer

Q2. A 32-year-old woman in her third session of psychotherapy becomes visibly withdrawn and gives only vague, one-word responses after the therapist suggests exploring her relationship with her mother. According to Safran and Muran, this is best described as:

A. A confrontation rupture

B. A withdrawal rupture

C. Negative transference

D. Resistance through secondary gain

▶ Answer

Q3. Jerome Frank and Julia Frank identified four features shared by all effective healing relationships. Which of the following is NOT one of their four features?

A. An emotionally charged, confiding relationship with a helping person

B. A healing setting

C. A manualized treatment protocol

D. A ritual or procedure requiring active participation of both patient and healer

▶ Answer

Q4. Bordin's (1979) pantheoretical model of the working alliance comprises three components. Which of the following correctly lists all three?

A. Empathy, unconditional positive regard, congruence

B. Goals, tasks, bond

C. Transference, countertransference, resistance

D. Hope, relationship, technique

▶ Answer

Q5. Truax and Carkhuff's (1967) landmark review of therapist effectiveness found that patients of therapists with LOW levels of empathy, warmth, and genuineness:

A. Showed no change compared to no treatment

B. Showed modest improvement

C. Deteriorated -- they got worse than without treatment

D. Improved at the same rate as patients of high-quality therapists

▶ Answer

Q6. In Wampold's Contextual Model, specific therapeutic techniques account for approximately what percentage of outcome variance?

A. 1%

B. 15%

C. 30%

D. 40%

▶ Answer

Q7. Which of the following correctly describes Rogers' six necessary and sufficient conditions for therapeutic personality change?

A. Three conditions focus on the therapist, three on the client

B. Four conditions focus on the therapist, two on the client

C. Two conditions describe the relational context, three the therapist-offered conditions, and one the client's perception

D. All six conditions describe therapist qualities

▶ Answer

Q8. The CASE Approach (Chronological Assessment of Suicide Events) developed by Shea is best described as:

A. A risk factor checklist for predicting suicide

B. A structured data-gathering interview method for eliciting suicidal ideation

C. A standardised decision-making tool for determining hospitalisation

D. A self-report questionnaire for measuring suicide risk

▶ Answer

Q9. Shea's validity technique of "gentle assumption" involves:

A. Asking about specific items rather than categories

B. Setting the upper limit very high so that minimisation still reveals pathology

C. Assuming the behaviour is already occurring in the phrasing of the question

D. Referencing what others experience to reduce stigma

▶ Answer

Q10. Eysenck's (1952) provocative claim about psychotherapy stated that:

A. Psychoanalysis was superior to all other forms of therapy

B. Approximately two-thirds of neurotic patients improved whether they received therapy or not

C. Behaviour therapy was the only effective form of psychological treatment

D. The therapeutic alliance was more important than specific techniques

▶ Answer

SECTION B: PSYCHODYNAMIC THERAPY (Q11-Q25)

Q11. A 28–year-old woman describes her new therapist as "the most brilliant doctor I've ever met -- she truly understands me" in session 2. By session 8, she says "this therapy is useless and you're just like every other incompetent doctor." The defence mechanism MOST evident here is:

A. Projection

B. Reaction formation

C. Splitting

D. Displacement

▶ Answer

Q12. A patient calmly and eloquently describes being physically beaten by his father throughout childhood, using clinical terminology and quoting statistics on child abuse prevalence, with no discernible emotion. This defence mechanism is best identified as:

A. Repression

B. Intellectualisation

C. Denial

D. Sublimation

▶ Answer

Q13. In Kernberg's structural model, the key diagnostic criterion that distinguishes borderline personality organisation from neurotic personality organisation is:

A. Presence of psychotic symptoms

B. Identity diffusion

C. Impaired reality testing

D. Use of projection

▶ Answer

Q14. Kohut's three selfobject transferences are:

A. Positive, negative, and erotic

B. Mirroring, idealising, and twinship

C. Concordant, complementary, and projective

D. Dependent, fight-flight, and pairing

▶ Answer

Q15. According to Gabbard, the intervention hierarchy from most expressive to most supportive is:

A. Empathic validation --> Clarification --> Confrontation --> Interpretation

B. Interpretation --> Confrontation --> Clarification --> Empathic validation

C. Interpretation --> Observation --> Confrontation --> Clarification --> Encouragement to elaborate --> Empathic validation --> Advice/Praise

D. Advice --> Empathic validation --> Interpretation --> Confrontation

▶ Answer

Q16. Racker's distinction between concordant and complementary countertransference is:

A. Concordant = therapist identifies with patient's projected object representation; complementary = therapist identifies with patient's self-representation

B. Concordant = therapist identifies with patient's projected self-representation; complementary = therapist identifies with patient's projected object representation

C. Concordant = positive feelings; complementary = negative feelings

D. Concordant = conscious awareness; complementary = unconscious enactment

▶ Answer

Q17. Projective identification differs from simple projection in that:

A. Projective identification involves conscious awareness of the projection

B. Projective identification includes interpersonal pressure that actually changes the other person's experience

C. Projection is a mature defence while projective identification is immature

D. Projection involves only positive feelings while projective identification involves negative ones

▶ Answer

Q18. The "triangle of insight" (Menninger/Malan) connects patterns across which three relational domains?

A. Past self, present self, future self

B. Transference relationship, current relationships, past relationships

C. Conscious, preconscious, unconscious

D. Id, ego, superego

▶ Answer

Q19. The Wallerstein (1986) Menninger Psychotherapy Research Project found that:

A. Expressive therapy was consistently superior to supportive therapy

B. Supportive treatment produced as much structural change as expressive therapy

C. Psychoanalysis was the only treatment that produced lasting change

D. Brief therapy was equivalent to long-term therapy for all conditions

▶ Answer

Q20. In Klein's object relations theory, the depressive position is characterised by:

A. Splitting, projective identification, and persecutory anxiety

B. Integration of good and bad objects, guilt, and the capacity for reparation

C. Identity diffusion and primitive idealisation

D. Primary narcissism and autoeroticism

▶ Answer

Q21. Winnicott's concept of the "false self" develops when:

A. The child identifies with the aggressor

B. The mother substitutes her own gesture for the infant's spontaneous gesture

C. The child represses unacceptable drives

D. The child fails to resolve the Oedipus complex

▶ Answer

Q22. A patient consciously decides not to think about a stressful work situation until after the weekend, saying "I know I'm worried, but I'll deal with it Monday." This is an example of:

A. Repression

B. Denial

C. Suppression

D. Dissociation

▶ Answer

Q23. Which statement about Freud's structural model is correct?

A. It replaced the topographic model because the topographic model could not explain unconscious defences

B. The id operates according to the reality principle

C. The superego is entirely conscious

D. The ego is entirely unconscious

▶ Answer

Q24. According to Gabbard, when should transference be interpreted?

A. As early as possible to demonstrate psychodynamic skill

B. Only when the transference becomes a resistance

C. Never -- transference should only be observed and contained

D. Only during the termination phase

▶ Answer

Q25. "Flight into health" during the termination phase of psychodynamic therapy typically represents:

A. Genuine therapeutic improvement

B. A resistance function -- premature claim of cure to avoid further exploration

C. The natural endpoint of successful therapy

D. A sign that the patient is ready for discharge

▶ Answer

SECTION C: COGNITIVE BEHAVIOUR THERAPY (Q26-Q45)

Q26. A patient thinks "I did well on that exam, but that was just luck -- it doesn't mean I'm actually competent." The cognitive distortion MOST evident is:

A. Emotional reasoning

B. Disqualifying the positive

C. Mind reading

D. Catastrophising

▶ Answer

Q27. In Beck's cognitive model, the three levels of cognition from most accessible to least accessible are:

A. Core beliefs, intermediate beliefs, automatic thoughts

B. Schemas, core beliefs, automatic thoughts

C. Automatic thoughts, intermediate beliefs, core beliefs

D. Automatic thoughts, core beliefs, intermediate beliefs

▶ Answer

Q28. Beck's cognitive triad for depression consists of a negative view of:

A. Past, present, and future

B. Self, world/experience, and future

C. Self, others, and relationships

D. Thoughts, feelings, and behaviour

▶ Answer

Q29. The correct standard structure of a CBT session is:

A. Free association, interpretation, working through, summary

B. Mood check, agenda setting, homework review, work on agenda items, summary, new homework, feedback

C. Mindfulness exercise, skills teaching, role play, homework

D. Check-in, unstructured exploration, psychoeducation, relaxation

▶ Answer

Q30. Collaborative empiricism in CBT is best described as:

A. The therapist challenges the patient's irrational beliefs through logical disputation

B. Therapist and client function as co-investigators examining evidence for and against beliefs

C. The therapist assigns homework and the patient reports back

D. The therapist teaches the patient about cognitive distortions through psychoeducation

▶ Answer

Q31. A patient with social anxiety avoids eye contact, rehearses what to say before speaking, and holds a glass tightly to hide hand tremor at parties. In the Clark and Wells model, these are examples of:

A. Cognitive distortions

B. Safety behaviours

C. Schema maintenance processes

D. Experiential avoidance

▶ Answer

Q32. The "downward arrow technique" in CBT is used to:

A. Reduce the intensity of automatic thoughts

B. Identify underlying intermediate and core beliefs from automatic thoughts

C. Create a fear hierarchy for graded exposure

D. Teach relaxation skills progressively

▶ Answer

Q33. According to Beck, negative core beliefs about the self fall into which three categories?

A. Anxiety, depression, anger

B. Abandonment, mistrust, defectiveness

C. Helplessness, unlovability, worthlessness

D. Id, ego, superego

▶ Answer

Q34. Behavioural experiments in CBT are considered more powerful than verbal techniques alone because they:

A. Are easier for the patient to complete

B. Provide direct experiential evidence against distorted beliefs

C. Do not require the therapeutic alliance

D. Work faster than medication

▶ Answer

Q35. A patient says "I feel incompetent, therefore I must be incompetent." This cognitive distortion is:

A. All-or-nothing thinking

B. Personalisation

C. Emotional reasoning

D. Overgeneralisation

▶ Answer

Q36. Clark's (1986) cognitive model of panic disorder centres on:

A. Conditioned fear responses to bodily sensations

B. Catastrophic misinterpretation of bodily sensations

C. Intolerance of uncertainty

D. Inflated responsibility for harm prevention

▶ Answer

Q37. In the Salkovskis (1985) cognitive model of OCD, the key pathological appraisal that distinguishes clinical obsessions from normal intrusive thoughts is:

A. Thought-action fusion

B. Inflated personal responsibility for preventing harm

C. Intolerance of uncertainty

D. Perfectionism

▶ Answer

Q38. All of the following are cognitive distortions EXCEPT:

A. Emotional reasoning

B. Collaborative empiricism

C. Mental filter

D. Personalisation

▶ Answer

Q39. The CBT-E (Enhanced CBT) model for eating disorders is considered "transdiagnostic" because:

A. It uses techniques from multiple therapy modalities

B. It can be applied to AN, BN, and BED using the same framework

C. It requires no diagnosis before treatment begins

D. It treats comorbid depression and anxiety simultaneously

▶ Answer

Q40. A patient says "My boss looked at me briefly during the meeting and then looked away. He must think my presentation was terrible." This cognitive distortion is:

A. Catastrophising

B. Mind reading

C. All-or-nothing thinking

D. Labelling

▶ Answer

Q41. The Abstinence Violation Effect (AVE) in CBT relapse prevention for substance use disorders refers to:

A. The physical withdrawal symptoms that occur after stopping substance use

B. The patient's attribution of a lapse to internal, stable, global causes, catastrophising it into full relapse

C. The therapist's refusal to continue treatment after a relapse

D. The mandatory period of abstinence required before starting therapy

▶ Answer

Q42. Which of the following is the FIRST intervention typically used in CBT for depression?

A. Cognitive restructuring of core beliefs

B. Behavioural activation and activity scheduling

C. Schema change work

D. Socratic questioning of intermediate beliefs

▶ Answer

Q43. In Beck's cognitive model, what distinguishes a schema from a core belief?

A. They are the same thing

B. A schema is the cognitive structure; core beliefs are the content of schemas

C. Core beliefs are more accessible than schemas

D. Schemas are conscious while core beliefs are unconscious

▶ Answer

Q44. Applied tension (Ost & Sterner, 1987) is the treatment of choice for which specific phobia?

A. Claustrophobia

B. Social phobia

C. Blood-injection-injury phobia

D. Agoraphobia

▶ Answer

Q45. Which key CBT trial found that CBT was less effective than IPT and imipramine for severely depressed patients?

A. Rush et al. (1977)

B. NIMH Treatment of Depression Collaborative Research Program (Elkin et al., 1989)

C. CoBalT trial (Wiles et al., 2013)

D. TADS Study (2004)

▶ Answer

SECTION D: BEHAVIOUR THERAPY (Q46-Q55)

Q46. Systematic desensitisation, developed by Wolpe (1958), is based on the principle of:

A. Operant conditioning

B. Reciprocal inhibition

C. Classical extinction

D. Cognitive restructuring

▶ Answer

Q47. In operant conditioning, a token economy is an example of:

A. Negative reinforcement

B. Positive punishment

C. Positive reinforcement

D. Negative punishment

▶ Answer

Q48. In the context of exposure therapy, which statement about safety behaviours is correct?

A. Safety behaviours enhance the effectiveness of exposure

B. Safety behaviours maintain anxiety by preventing disconfirmation of feared outcomes

C. Safety behaviours should be encouraged during initial exposure sessions

D. Safety behaviours have no effect on exposure outcomes

▶ Answer

Q49. Behavioural Activation (BA) for depression is based on which theoretical model?

A. Beck's cognitive triad

B. Lewinsohn's model of reduced positive reinforcement

C. Seligman's learned helplessness

D. Bandura's self-efficacy theory

▶ Answer

Q50. The inhibitory learning model of exposure therapy proposes that:

A. The original fear memory is erased through repeated exposure

B. A new non-threat association is formed alongside the existing fear memory

C. Relaxation inhibits the anxiety response

D. Cognitive restructuring is required for exposure to work

▶ Answer

Q51. Exposure and Response Prevention (ERP) for OCD specifically requires:

A. Exposure to feared stimuli with relaxation training

B. Exposure to obsessional triggers with absolute prevention of compulsive rituals

C. Imaginal exposure only, without in vivo components

D. Brief exposure followed by immediate distraction

▶ Answer

Q52. In Contingency Management (CM) for substance use disorders, the primary mechanism is:

A. Cognitive restructuring of beliefs about substance use

B. Providing tangible reinforcers for verified abstinence

C. Exposure and desensitisation to drug cues

D. Interpersonal skills training

▶ Answer

Q53. Single-session exposure treatment (Ost, 1989) has been shown to produce clinically significant improvement in:

A. Social anxiety disorder

B. Specific phobias

C. PTSD

D. Generalised anxiety disorder

▶ Answer

Q54. The MANAS trial (Patel et al., 2010) demonstrated that:

A. CBT was superior to medication for depression in India

B. Collaborative stepped care including BA delivered by lay counsellors was effective for common mental disorders

C. Psychodynamic therapy could be adapted for low-resource settings

D. Schema Therapy was effective for chronic depression in developing countries

▶ Answer

Q55. In the Community Reinforcement Approach (CRA) for substance use, the primary mechanism is:

A. Confronting denial about substance use

B. Rearranging environmental contingencies so sobriety is more rewarding than substance use

C. Teaching cognitive restructuring of substance-related thoughts

D. Prescribing aversive consequences for substance use

▶ Answer

SECTION E: DBT (Q56-Q70)

Q56. The biosocial model of DBT explains emotional dysregulation as the transaction between:

A. Cognitive distortions and behavioural avoidance

B. Biological vulnerability to intense emotions and an invalidating environment

C. Attachment insecurity and mentalization failure

D. Early maladaptive schemas and maladaptive coping modes

▶ Answer

Q57. In DBT's target hierarchy for Stage 1 individual therapy, which behaviour is addressed FIRST?

A. Quality-of-life-interfering behaviours

B. Therapy-interfering behaviours

C. Life-threatening behaviours

D. Increasing behavioural skills

▶ Answer

Q58. The TIPP skills in DBT's Distress Tolerance module stands for:

A. Think, Identify, Plan, Practice

B. Temperature, Intense exercise, Paced breathing, Progressive relaxation

C. Trust, Improve, Persist, Participate

D. Think, Imagine, Problem-solve, Proceed

▶ Answer

Q59. DEAR MAN in DBT is used for:

A. Maintaining self-respect in interpersonal situations

B. Getting what you want or saying no effectively

C. Maintaining the relationship during conflict

D. Reducing hostile attributions toward others

▶ Answer

Q60. In DBT, which is the correct application of opposite action?

A. When feeling fear, the opposite action is to avoid the feared situation

B. When feeling unjustified shame, the opposite action is to hide and withdraw

C. When feeling sadness, the opposite action is to become active and engage

D. When feeling justified guilt, the opposite action is to refuse to apologise

▶ Answer

Q61. The four modes of standard DBT treatment are:

A. Individual therapy, group therapy, family therapy, medication management

B. Individual therapy, skills group, phone coaching, consultation team

C. Assessment, stabilisation, trauma processing, integration

D. Psychoeducation, skills training, exposure, relapse prevention

▶ Answer

Q62. In DBT, the formula "Pain + Non-acceptance = Suffering" describes which skill?

A. Opposite action

B. Radical acceptance

C. Wise Mind

D. Check the Facts

▶ Answer

Q63. Walking the Middle Path is a DBT skills module unique to:

A. Standard adult DBT

B. DBT for substance use disorders

C. Adolescent DBT (Rathus & Miller)

D. DBT for eating disorders

▶ Answer

Q64. In DBT, which level of validation involves treating the person as a capable equal with honest, direct, warm communication?

A. Level 3 -- Mind-reading

B. Level 4 -- Validation by past history

C. Level 5 -- Validation by current context

D. Level 6 -- Radical genuineness

▶ Answer

Q65. The three dialectical dilemmas in standard adult DBT (Linehan) are:

A. Acceptance vs. change, validation vs. confrontation, mindfulness vs. action

B. Emotional vulnerability vs. self-invalidation, active passivity vs. apparent competence, unrelenting crisis vs. inhibited experiencing

C. Attachment vs. autonomy, emotion vs. reason, past vs. present

D. Biological vs. environmental, internal vs. external, individual vs. systemic

▶ Answer

Q66. In DBT chain analysis, the correct sequence of steps is:

A. Identify the emotion, then the thought, then the behaviour

B. Identify problem behaviour, prompting event, vulnerability factors, then map the chain link by link

C. Identify the core belief, then the intermediate belief, then the automatic thought

D. Identify the schema, then the mode, then the coping style

▶ Answer

Q67. Linehan et al. (2006) found that DBT for BPD women was superior to community treatment by experts in all of the following EXCEPT:

A. Halving suicide attempts

B. Reducing self-harm

C. Curing personality disorder

D. Reducing ED visits and psychiatric hospitalisation

▶ Answer

Q68. The Wise Mind concept in DBT is:

A. The same as the Adult ego state in Transactional Analysis

B. The synthesis of Emotional Mind and Reasonable Mind; intuitive knowing

C. A cognitive restructuring technique borrowed from CBT

D. The therapist's perspective on the patient's problems

▶ Answer

Q69. ABC PLEASE in DBT's Emotion Regulation module includes all of the following EXCEPT:

A. Accumulate positives

B. Avoid mood-altering substances

C. Analyse cognitive distortions

D. Build mastery

▶ Answer

Q70. In DBT, when a patient does not complete the diary card, this is classified as:

A. A life-threatening behaviour

B. A therapy-interfering behaviour

C. A quality-of-life-interfering behaviour

D. Not clinically significant

▶ Answer

SECTION F: SCHEMA THERAPY (Q71-Q80)

Q71. According to Young, the 18 Early Maladaptive Schemas are grouped into how many domains?

A. 3

B. 4

C. 5

D. 6

▶ Answer

Q72. In Schema Therapy, behaviour is:

A. Part of the schema itself

B. NOT part of the schema -- it is a coping response to the schema

C. The primary target of treatment

D. Irrelevant to the formulation

▶ Answer

Q73. Limited reparenting in Schema Therapy involves:

A. The therapist becoming a substitute parent for the patient

B. The therapist providing, within appropriate boundaries, what the patient needed but did not receive from parents

C. Teaching parenting skills to the patient

D. Inviting the patient's actual parents into therapy sessions

▶ Answer

Q74. In Schema Therapy imagery rescripting, the therapist enters the childhood scene as a helping figure. The three types of helping figures, in order of increasing patient Healthy Adult strength, are:

A. Parent figure, therapist, patient's own Healthy Adult

B. Therapist enters, third-party helper, patient as own Healthy Adult

C. Patient's own Healthy Adult, third-party helper, therapist enters

D. God/spiritual figure, therapist, patient

▶ Answer

Q75. The Giesen-Bloo et al. (2006) RCT comparing Schema Therapy and TFP for BPD found:

A. TFP was superior to Schema Therapy

B. Both treatments were equally effective

C. Schema Therapy showed higher recovery rates and lower dropout

D. Neither treatment was effective

▶ Answer

Q76. The four core BPD modes in the Schema Therapy mode model are:

A. Vulnerable Child, Angry Child, Punitive Parent, Healthy Adult

B. Abandoned/Abused Child, Angry/Impulsive Child, Punitive Parent, Detached Protector

C. Happy Child, Compliant Surrenderer, Overcompensator, Healthy Adult

D. Lonely Child, Demanding Parent, Self-Aggrandiser, Detached Self-Soother

▶ Answer

Q77. Which of the following is TRUE about imagery rescripting in Schema Therapy?

A. It is a form of prolonged exposure therapy

B. It aims for complete emotional habituation to traumatic memories

C. It is NOT exposure therapy -- the patient contacts emotion but does not relive full trauma

D. It should only be used in the first few sessions of treatment

▶ Answer

Q78. A patient with a Defectiveness/Shame schema who constantly strives for perfection and achievement to prove they are worthy of love is demonstrating which coping style?

A. Surrender

B. Avoidance

C. Overcompensation

D. Sublimation

▶ Answer

Q79. According to Young, the five core emotional needs that, when unmet, lead to schema development are:

A. Food, shelter, safety, love, autonomy

B. Secure attachment, autonomy/competence, freedom to express needs, spontaneity/play, realistic limits

C. Belonging, esteem, self-actualisation, safety, physiological needs

D. Trust, identity, intimacy, generativity, integrity

▶ Answer

Q80. In the Arntz/Bernstein expanded mode model, the Detached Self-Soother mode involves:

A. Healthy self-care and relaxation

B. Addictive soothing or stimulation-seeking behaviour

C. Turning to others for emotional support

D. Rational problem-solving

▶ Answer

SECTION G: HUMANISTIC-EXISTENTIAL & MI (Q81-Q90)

Q81. Yalom identified four "ultimate concerns" of existence that form the basis of existential psychotherapy. These are:

A. Love, work, play, and meaning

B. Death, freedom, isolation, and meaninglessness

C. Anxiety, guilt, shame, and despair

D. Being, nothingness, authenticity, and transcendence

▶ Answer

Q82. In Motivational Interviewing (MI), the acronym DARN-CAT represents types of:

A. Resistance statements

B. Change talk

C. Cognitive distortions

D. Therapeutic interventions

▶ Answer

Q83. Rogers argued that three "therapist-offered" conditions are necessary and sufficient for therapeutic change. In which situation would a Rogerian therapist be MOST concerned?

A. The patient does not agree with the therapist's formulation

B. The patient perceives the therapist as judgmental despite the therapist's efforts

C. The patient is resistant to homework assignments

D. The patient has a poor understanding of their diagnosis

▶ Answer

Q84. The empty chair technique is commonly associated with Gestalt therapy (Perls), but was originally developed by:

A. Sigmund Freud

B. Carl Rogers

C. Jacob Moreno

D. Aaron Beck

▶ Answer

Q85. The four processes of Motivational Interviewing (Miller & Rollnick, 2013) are:

A. Assessment, formulation, intervention, evaluation

B. Engaging, focusing, evoking, planning

C. Contemplation, preparation, action, maintenance

D. Joining, enactment, restructuring, termination

▶ Answer

Q86. Gestalt therapy, developed by Fritz Perls, emphasises:

A. Free association and dream interpretation

B. Awareness, present-moment experience, and personal responsibility

C. Systematic desensitisation and reciprocal inhibition

D. Cognitive restructuring and behavioural experiments

▶ Answer

Q87. Project MATCH (1997) for alcohol dependence compared three treatments and found:

A. CBT was clearly superior to all other treatments

B. Twelve-Step Facilitation, CBT, and MET were broadly equivalent at 1–year and 3–year follow-up

C. MET was ineffective

D. Only medication produced lasting improvement

▶ Answer

Q88. Abraham Maslow's contribution to psychotherapy included:

A. The cognitive triad for depression

B. The hierarchy of needs and the concept of self-actualisation

C. The structural model of personality (id, ego, superego)

D. The three waves of behaviour therapy

▶ Answer

Q89. In existential psychotherapy, "existential guilt" arises from:

A. Violating the superego's moral standards

B. Failing to live authentically -- not actualising one's potential

C. Unresolved Oedipal conflicts

D. Cognitive distortions about personal responsibility

▶ Answer

Q90. Harry Stack Sullivan's interpersonal psychiatry emphasised that:

A. The unconscious is the primary driver of all behaviour

B. Personality is the relatively enduring pattern of recurrent interpersonal situations

C. Self-actualisation is the primary human motivation

D. Operant conditioning shapes all human behaviour

▶ Answer

SECTION H: SPECIALIZED MODALITIES (Q91-Q100)

Q91. The four problem areas in Interpersonal Therapy (IPT) are:

A. Trauma, loss, conflict, and isolation

B. Grief, role disputes, role transitions, and interpersonal deficits

C. Depression, anxiety, personality, and substance use

D. Family, work, social, and intimate relationships

▶ Answer

Q92. The eight phases of EMDR are, in order:

A. Assessment, preparation, desensitisation, installation, body scan, closure, re-evaluation, history

B. History, preparation, assessment, desensitisation, installation, body scan, closure, re-evaluation

C. Preparation, history, assessment, processing, integration, body scan, consolidation, follow-up

D. Engagement, stabilisation, processing, installation, body scan, integration, termination, follow-up

▶ Answer

Q93. In Mentalization-Based Treatment (MBT), "psychic equivalence" is a pre-mentalizing mode in which:

A. Mental states are decoupled from reality

B. Inner and outer reality are equated -- what I feel IS what is real

C. Mental states can only be understood through observable actions

D. The patient can reflect on their own mental states accurately

▶ Answer

Q94. Kernberg's TFP treatment hierarchy places which concern at the TOP priority?

A. Narcissistic transference developments

B. Contract violations

C. Threats to the patient's life

D. Material discussed in a blocked manner

▶ Answer

Q95. In EFT for couples (Johnson), the "softening" is:

A. The therapist becoming gentler with the withdrawing partner

B. The pursuing partner expressing underlying vulnerability rather than anger and reaching for the withdrawing partner

C. The withdrawing partner becoming more emotionally expressive

D. Both partners agreeing to avoid conflict

▶ Answer

Q96. In Transactional Analysis (Berne), an "ulterior transaction" is one where:

A. The response comes from an unexpected ego state

B. Two messages are sent simultaneously -- a social (overt) and a psychological (covert) message

C. Communication breaks down due to a crossed transaction

D. The Adult ego state is bypassed

▶ Answer

Q97. The Bateman & Fonagy (2008) 8–year follow-up of MBT for BPD found that:

A. All gains were lost at follow-up

B. 13% of MBT patients still met BPD criteria vs. 87% in treatment as usual

C. MBT and TAU groups converged over time

D. MBT patients had higher rates of substance use at follow-up

▶ Answer

Q98. Psychodrama's core technique of "role reversal" involves:

A. The therapist taking the patient's role

B. The protagonist switching roles with the auxiliary ego, playing the other person

C. All group members switching seats

D. The patient playing a different character in a scripted play

▶ Answer

Q99. The "sick role" in IPT is borrowed from Talcott Parsons and serves which function?

A. Labelling the patient as permanently disabled

B. Reducing self-blame and simultaneously obligating the patient to work toward recovery

C. Excusing the patient from all responsibility

D. Justifying long-term disability benefits

▶ Answer

Q100. Supportive psychotherapy is best described as:

A. The absence of technique -- "just chatting"

B. A deliberate, structured approach aimed at symptom relief and ego strengthening without uncovering unconscious material

C. An inferior form of therapy used only when "real therapy" is unavailable

D. A technique used only in inpatient settings

▶ Answer

SECTION I: GROUP THERAPY (Q101-Q105)

Q101. Yalom's 11 therapeutic factors in group therapy include all of the following EXCEPT:

A. Universality

B. Transference interpretation

C. Interpersonal learning

D. Group cohesiveness

▶ Answer

Q102. Bion's three basic assumption groups are:

A. Fight, flight, and freeze

B. Dependency, fight-flight, and pairing

C. Forming, storming, and norming

D. Id, ego, and superego

▶ Answer

Q103. The most commonly cited contraindication for outpatient process group therapy is:

A. Social anxiety disorder

B. Major depressive disorder

C. Severe antisocial personality disorder

D. Generalised anxiety disorder

▶ Answer

Q104. In Tuckman's model of group development, the phase where most premature dropouts occur is:

A. Forming

B. Storming

C. Norming

D. Performing

▶ Answer

Q105. Lieberman, Yalom, and Miles (1973) found that the LEAST effective group leaders were those characterised by:

A. High caring and moderate stimulation

B. High emotional stimulation with low caring ("aggressive stimulators")

C. Moderate executive function and high meaning attribution

D. Low emotional stimulation with high caring

▶ Answer

SECTION J: FAMILY & COUPLES THERAPY (Q106-Q115)

Q106. Gottman's "Four Horsemen of the Apocalypse" are:

A. Anger, sadness, fear, disgust

B. Criticism, contempt, defensiveness, stonewalling

C. Avoidance, pursuit, attack, withdrawal

D. Enmeshment, disengagement, triangulation, scapegoating

▶ Answer

Q107. Gottman's research found that stable, happy marriages maintain a positive-to-negative interaction ratio during conflict of at least:

A. 1:1

B. 3:1

C. 5:1

D. 10:1

▶ Answer

Q108. Bowen's concept of "differentiation of self" is best defined as:

A. Separating from one's family of origin completely

B. The capacity to maintain one's own thinking and emotional functioning in the face of family pressure

C. Developing a separate identity during adolescence

D. Achieving financial independence from parents

▶ Answer

Q109. In structural family therapy, the technique of "enactment" involves:

A. The therapist telling the family what they are doing wrong

B. Asking family members to demonstrate their typical interaction pattern in session

C. Prescribing the symptom paradoxically

D. Drawing a genogram of three generations

▶ Answer

Q110. The "double bind" concept was originally proposed by:

A. Minuchin

B. Bowen

C. Bateson, Jackson, Haley & Weakland

D. Gottman

▶ Answer

Q111. In Gottman's Sound Relationship House model, which component forms the BASE?

A. Managing conflict

B. Trust and commitment

C. Love Maps

D. Creating shared meaning

▶ Answer

Q112. Minuchin proposed that enmeshed families produce psychosomatic symptoms. His "psychosomatic family" model identifies four characteristics. Which of the following is NOT one of them?

A. Enmeshment

B. Overprotectiveness

C. High expressed emotion

D. Lack of conflict resolution

▶ Answer

Q113. In strategic family therapy, a paradoxical intervention works because:

A. It confuses the family into changing

B. If the patient complies, they gain control over the symptom; if they rebel, the symptom disappears

C. It always makes the family angry, which motivates change

D. It bypasses the patient's cognitive distortions

▶ Answer

Q114. According to Gottman, what percentage of marital problems are "perpetual" (fundamental differences that will never be fully resolved)?

A. 10%

B. 31%

C. 50%

D. 69%

▶ Answer

Q115. Bowen's concept of "emotional cutoff" refers to:

A. A healthy boundary between family members

B. Managing unresolved emotional issues by reducing or totally cutting off family contact

C. The therapist ending a session early

D. A technique for managing conflict in couples

▶ Answer

SECTION K: DISORDER-SPECIFIC THERAPY MAP (Q116-Q125)

Q116. According to NICE guidelines, which psychological therapy is first-line for relapse prevention in recurrent depression (3+ episodes)?

A. CBT

B. IPT

C. MBCT

D. Schema Therapy

▶ Answer

Q117. For adolescent anorexia nervosa, the first-line psychotherapy recommended by NICE is:

A. CBT-E

B. Family-Based Treatment (FBT / Maudsley approach)

C. IPT

D. Psychodynamic therapy

▶ Answer

Q118. The CoBalT trial (Wiles et al., 2013) demonstrated that:

A. CBT alone is superior to antidepressants for severe depression

B. CBT added to antidepressants improved outcomes in treatment-resistant depression with an NNT of 4

C. IPT is superior to CBT for treatment-resistant depression

D. Medication is the only effective treatment for treatment-resistant depression

▶ Answer

Q119. For Post-Traumatic Stress Disorder, NICE recommends which treatments as first-line?

A. Supportive counselling and relaxation therapy

B. Trauma-focused CBT (PE, CPT) and EMDR

C. Schema Therapy and DBT

D. IPT and psychodynamic therapy

▶ Answer

Q120. The TADS Study (2004) for adolescent depression found that the treatment with the HIGHEST response rate was:

A. CBT alone

B. Fluoxetine alone

C. CBT + fluoxetine combined

D. Placebo

▶ Answer

Q121. According to NICE, psychological intervention for schizophrenia should include CBTp offered for at least how many planned sessions?

A. 6

B. 10

C. 16

D. 24

▶ Answer

Q122. The social zeitgeber theory, which underpins IPSRT for bipolar disorder, proposes that:

A. Social isolation causes mania

B. Disruptions to social rhythms destabilise circadian rhythms, triggering mood episodes

C. Social conflict is the primary cause of depressive episodes

D. Social media use triggers bipolar cycling

▶ Answer

Q123. For BPD, NICE recommends against brief psychological interventions lasting less than:

A. 1 month

B. 3 months

C. 6 months

D. 12 months

▶ Answer

Q124. The Expressed Emotion (EE) model in schizophrenia identifies which three components that predict relapse?

A. Criticism, hostility, and emotional over-involvement

B. Anger, contempt, and withdrawal

C. Denial, projection, and splitting

D. Anxiety, depression, and psychosis

▶ Answer

Q125. Which statement about the MTA Study (1999) for ADHD is correct?

A. Behavioural treatment was superior to medication at all time points

B. At 14 months, carefully titrated medication was superior to intensive behavioural treatment for core ADHD symptoms

C. Combined treatment showed no advantages over medication alone

D. No treatment was effective for ADHD

▶ Answer

SECTION L: LANDMARK PAPERS (Q126-Q130)

Q126. The NIMH Treatment of Depression Collaborative Research Program (Elkin et al., 1989) compared which four treatment arms?

A. CBT, DBT, medication, placebo

B. CBT, IPT, imipramine + clinical management, pill placebo + clinical management

C. CBT, Schema Therapy, fluoxetine, waiting list

D. Psychoanalysis, CBT, medication, no treatment

▶ Answer

Q127. Fluckiger et al. (2018) conducted the largest meta-analysis of the therapeutic alliance and found an alliance-outcome correlation of:

A. r = 0.05 across 50 studies

B. r = 0.278 across 295 independent studies

C. r = 0.80 across 10 studies

D. r = 0.50 across 150 studies

▶ Answer

Q128. Dimidjian et al. (2006) found that Behavioural Activation for depression was:

A. Inferior to both CBT and medication

B. As effective as antidepressant medication and superior to CBT for severe depression

C. Only effective for mild depression

D. Effective only when combined with medication

▶ Answer

Q129. The Colom et al. (2003, 2009) studies demonstrated that group psychoeducation for bipolar disorder:

A. Had no effect on relapse

B. Significantly reduced relapse rates at 2–year and 5–year follow-up

C. Was inferior to CBT

D. Only worked when delivered individually

▶ Answer

Q130. Linehan et al. (1991) -- the first RCT of DBT for BPD -- found DBT was superior to treatment as usual in all of the following EXCEPT:

A. Fewer suicide attempts

B. Less self-harm

C. Fewer hospitalisations

D. Complete remission of personality disorder in all patients

▶ Answer

Compiled for the Weave Psychotherapy series. Sources: WP-02 (Foundations), WP-03 (Psychodynamic), WP-04 (CBT), WP-06 (Third Wave), WP-08 (Specialized Modalities), WP-09 (Group/Family/Couples), WP-11 (Disorder-Specific Map).

02

Clinical Quick Reference

Psychotherapy Question Bank — Weave Psychotherapy
Vol. 13

D6: Psychotherapy Question Bank -- Answer Key & High-Yield Summary

KEY INSIGHT

Companion to: D1–QuestionBank-DeepStudy.md (130 MCQs)

1. QUICK ANSWER KEY

Q	Ans	Q	Ans	Q	Ans	Q	Ans	Q	Ans
1	B	27	C	53	B	79	B	105	B
2	B	28	B	54	B	80	B	106	B
3	C	29	B	55	B	81	B	107	C
4	B	30	B	56	B	82	B	108	B
5	C	31	B	57	C	83	B	109	B
6	A	32	B	58	B	84	C	110	C
7	C	33	C	59	B	85	B	111	C
8	B	34	B	60	C	86	B	112	C
9	C	35	C	61	B	87	B	113	B
10	B	36	B	62	B	88	B	114	D
11	C	37	B	63	C	89	B	115	B
12	B	38	B	64	D	90	B	116	C
13	B	39	B	65	B	91	B	117	B
14	B	40	B	66	B	92	B	118	B
15	C	41	B	67	C	93	B	119	B
16	B	42	B	68	B	94	C	120	C
17	B	43	B	69	C	95	B	121	C
18	B	44	C	70	B	96	B	122	B
19	B	45	B	71	C	97	B	123	B
20	B	46	B	72	B	98	B	124	A
21	B	47	C	73	B	99	B	125	B
22	C	48	B	74	B	100	B	126	B

Q	Ans	Q	Ans	Q	Ans	Q	Ans	Q	Ans
23	A	49	B	75	C	101	B	127	B
24	B	50	B	76	B	102	B	128	B
25	B	51	B	77	C	103	C	129	B
26	B	52	B	78	C	104	B	130	D

2. QUESTIONS SORTED BY TOPIC

Foundations & Common Factors (10 Qs)

Q1 (Lambert's model), Q2 (Safran ruptures), Q3 (Frank's four features), Q4 (Bordin's alliance), Q5 (Truax & Carkhuff), Q6 (Wampold), Q7 (Rogers' conditions), Q8 (CASE approach), Q9 (Gentle assumption), Q10 (Eysenck)

Psychodynamic Therapy (15 Qs)

Q11 (Splitting), Q12 (Intellectualisation), Q13 (Kernberg's organisation levels), Q14 (Kohut's selfobject transferences), Q15 (Intervention hierarchy), Q16 (Racker's CT types), Q17 (Projective identification), Q18 (Triangle of insight), Q19 (Wallerstein study), Q20 (Klein's positions), Q21 (Winnicott's false self), Q22 (Suppression), Q23 (Structural model), Q24 (When to interpret transference), Q25 (Flight into health)

CBT (20 Qs)

Q26 (Disqualifying the positive), Q27 (Three levels of cognition), Q28 (Cognitive triad), Q29 (CBT session structure), Q30 (Collaborative empiricism), Q31 (Safety behaviours), Q32 (Downward arrow), Q33 (Core belief categories), Q34 (Behavioural experiments), Q35 (Emotional reasoning), Q36 (Clark's panic model), Q37 (Salkovskis OCD model), Q38 (Identify non-distortion), Q39 (CBT-E transdiagnostic), Q40 (Mind reading), Q41 (Abstinence Violation Effect), Q42 (First intervention in CBT for depression), Q43 (Schema vs. core belief), Q44 (Applied tension for BII phobia), Q45 (NIMH TDCRP and CBT)

Behaviour Therapy (10 Qs)

Q46 (Systematic desensitisation), Q47 (Token economy), Q48 (Safety behaviours in exposure), Q49 (BA theoretical model), Q50 (Inhibitory learning), Q51 (ERP for OCD), Q52 (Contingency Management), Q53 (Single-session exposure), Q54 (MANAS trial), Q55 (CRA)

DBT (15 Qs)

Q56 (Biosocial model), Q57 (Target hierarchy), Q58 (TIPP), Q59 (DEAR MAN), Q60 (Opposite action), Q61 (Four modes of DBT), Q62 (Radical acceptance), Q63 (Walking the Middle Path), Q64 (Validation Level 6), Q65 (Dialectical dilemmas), Q66 (Chain analysis), Q67 (DBT outcomes), Q68 (Wise Mind), Q69 (ABC PLEASE), Q70 (Diary card non-completion)

Schema Therapy (10 Qs)

Q71 (5 domains), Q72 (Behaviour not part of schema), Q73 (Limited reparenting), Q74 (Imagery rescripting helpers), Q75 (Giesen-Bloo RCT), Q76 (BPD mode model), Q77 (IR is not exposure), Q78 (Overcompensation), Q79 (Five core emotional needs), Q80 (Detached Self-Soother)

Humanistic-Existential & MI (10 Qs)

Q81 (Yalom's ultimate concerns), Q82 (DARN-CAT change talk), Q83 (Rogers' perception condition), Q84 (Empty chair origins), Q85 (MI four processes), Q86 (Gestalt therapy), Q87 (Project MATCH), Q88 (Maslow), Q89 (Existential guilt), Q90 (Sullivan)

Specialized Modalities (10 Qs)

Q91 (IPT problem areas), Q92 (EMDR phases), Q93 (Psychic equivalence in MBT), Q94 (TFP hierarchy), Q95 (EFT softening), Q96 (TA ulterior transactions), Q97 (MBT 8–year follow-up), Q98 (Psychodrama role reversal), Q99 (Sick role in IPT), Q100 (Supportive psychotherapy)

Group Therapy (5 Qs)

Q101 (Yalom's 11 factors), Q102 (Bion's basic assumptions), Q103 (Contraindication: ASPD), Q104 (Storming = dropout), Q105 (Aggressive stimulators)

Family & Couples Therapy (10 Qs)

Q106 (Four Horsemen), Q107 (5:1 ratio), Q108 (Differentiation of self), Q109 (Enactment), Q110 (Double bind), Q111 (Sound Relationship House base), Q112 (Psychosomatic family), Q113 (Paradoxical intervention), Q114 (69% perpetual problems), Q115 (Emotional cutoff)

Disorder-Specific Map (10 Qs)

Q116 (MBCT for relapse prevention), Q117 (FBT for adolescent AN), Q118 (CoBalT trial), Q119 (PTSD first-line), Q120 (TADS study), Q121 (CBTp sessions), Q122 (Social zeitgeber theory), Q123 (BPD minimum duration), Q124 (Expressed Emotion), Q125 (MTA study)

Landmark Papers (5 Qs)

Q126 (NIMH TDCRP arms), Q127 (Fluckiger alliance meta-analysis), Q128 (Dimidjian BA study), Q129 (Colom psychoeducation), Q130 (Linehan 1991 RCT)

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3. TOP 20 MOST COMMONLY TESTED CONCEPTS

Rank	Concept	Source Volume	Key Fact
1	Lambert's 40–30–15–15 model	WP-02	Extratherapeutic 40%, relationship 30%, expectancy 15%, technique 15%
2	Vaillant's defence hierarchy	WP-03	Primitive (splitting, PI) --> Neurotic (repression, intellectualisation) --> Mature (sublimation, humour)
3	Beck's cognitive triad	WP-04	Negative view of self, world, future (specific to depression)
4	DBT target hierarchy	WP-06	Life-threatening > therapy-interfering > quality-of-life > skills
5	Cognitive distortions (all 12)	WP-04	All-or-nothing, catastrophising, emotional reasoning, mind reading most tested
6	Yalom's 11 therapeutic factors	WP-09	Group cohesiveness = alliance equivalent; interpersonal learning = key differentiator

Rank	Concept	Source Volume	Key Fact
7	Gottman's Four Horsemen	WP-09	Criticism, contempt, defensiveness, stonewalling; contempt = best divorce predictor
8	Rogers' core conditions (EUG)	WP-02	Empathy, unconditional positive regard, genuineness
9	Kernberg's personality organisation	WP-03	Neurotic vs. borderline vs. psychotic: identity, defences, reality testing
10	Schema Therapy 18 EMSs / 5 domains	WP-06	DISCO mnemonic; 5–4–2–3–4 distribution
11	IPT four problem areas (GRRD)	WP-08	Grief, role disputes, role transitions, interpersonal deficits
12	EMDR eight phases	WP-08	History, preparation, assessment, desensitisation, installation, body scan, closure, re-evaluation
13	Clark & Wells social anxiety model	WP-11	Self-focused attention, safety behaviours, post-event rumination

Rank	Concept	Source Volume	Key Fact
14	Biosocial model (DBT)	WP-06	Biological vulnerability x invalidating environment = dysregulation
15	Bordin's working alliance (GTB)	WP-02	Goals, tasks, bond -- pantheoretical
16	Three waves of CBT	WP-04/06	1st = behaviour, 2nd = cognition, 3rd = relationship with cognition
17	Bowen's differentiation of self	WP-09	Master concept; not autonomy -- capacity to be close without fusion
18	DBT interpersonal skills (DEAR MAN/GIVE/FAST)	WP-06	Objective, relationship, and self-respect effectiveness
19	BPD treatments head-to-head	WP-11	DBT (skills), MBT (mentalizing), TFP (object relations), ST (schemas)
20	NIMH TDCRP (Elkin 1989)	WP-08/11	Most important trial; CBT = IPT for mild-moderate; IPT > CBT for severe

4. COMMON TRAPS AND DISTRACTORS

Trap 1: "Cognition causes emotion" in CBT

The cognitive model claims cognition **mediates** between situations and emotions. It does NOT claim cognition causes emotion. This is a frequent exam trap.

Trap 2: DBT skills group = group therapy

DBT skills groups are **psychoeducational**, NOT process-oriented group therapy. Individual processing occurs in individual sessions, not in the skills group.

Trap 3: Suppression vs. repression

Suppression is **conscious** and a **mature** defence. Repression is **unconscious** and a **neurotic** defence. Both involve keeping something out of awareness, but the mechanism differs entirely.

Trap 4: Splitting vs. all-or-nothing thinking

Splitting is a **primitive defence mechanism** (psychodynamic concept) operating at the level of object relations. All-or-nothing thinking is a **cognitive distortion** (CBT concept) operating at the level of information processing. They describe similar phenomena from different theoretical frameworks.

Trap 5: Schema Therapy -- behaviour is NOT part of the schema

Schemas = memories + emotions + cognitions + bodily sensations. Behaviour is a **coping response** to the schema, not the schema itself. This is why the same schema produces opposite behaviours (surrender vs. overcompensation).

Trap 6: MBCT is NOT for acute depression

MBCT is specifically for **relapse prevention** in patients with 3+ prior episodes. It showed NO benefit for patients with 1–2 episodes (Teasdale et al., 2000).

Trap 7: Concordant vs. complementary countertransference

Concordant = therapist identifies with patient's **self**-representation (empathy). Complementary = therapist identifies with patient's **object**-representation (enacting the parent role). Easy to mix up; remember concordant = **con**-sonant with the patient's experience.

Trap 8: Projective identification vs. projection

Both are primitive defences. The key difference is **interpersonal pressure** -- in PI, the patient actually changes the other person's experience; in projection, the attribution exists only in the patient's mind.

Trap 9: Applied tension vs. relaxation for BII phobia

BII phobia uniquely involves a **parasympathetic** (vasovagal) response. Applied tension **increases** blood pressure. Relaxation would worsen the fainting response -- the opposite of what you want.

Trap 10: Frank's model vs. Lambert's model

Frank identified four **features** of all healing relationships (qualitative). Lambert identified four **factors** contributing to outcome variance with specific percentages (quantitative). They are complementary, not competing.

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5. SCORE INTERPRETATION GUIDE

Raw Score Bands (out of 130)

Score	Percentage	Interpretation	Recommended Action
117–130	90–100%	**Excellent.** Exam-ready across all topics.	Review any errors; focus on viva-style application
104–116	80–89%	**Strong.** Solid knowledge base with minor gaps.	Target weak sections; re-read relevant D1 chapters
91–103	70–79%	**Good.** Adequate for pass but room for improvement.	Systematic review of missed topics; focus on high-yield concepts
78–90	60–69%	**Borderline.** Pass possible but unreliable.	Intensive revision needed; prioritise top 20 concepts
65–77	50–59%	**Below pass.** Significant knowledge gaps.	Full re-read of D1 volumes; focus on foundations before details
<65	<50%	**Insufficient.** Major gaps across multiple domains.	Start from WP-02 (Foundations) and work systematically through each volume

Section-Level Analysis

For targeted revision, calculate your score per section and identify your two weakest areas:

Section	Questions	Out of	Your Score
A: Foundations	Q1–Q10	10	__
B: Psychodynamic	Q11–Q25	15	__
C: CBT	Q26–Q45	20	__
D: Behaviour Therapy	Q46–Q55	10	__
E: DBT	Q56–Q70	15	__
F: Schema Therapy	Q71–Q80	10	__
G: Humanistic/Existential/MI	Q81–Q90	10	__
H: Specialized Modalities	Q91–Q100	10	__
I: Group Therapy	Q101–Q105	5	__
J: Family & Couples	Q106–Q115	10	__
K: Disorder-Specific	Q116–Q125	10	__
L: Landmark Papers	Q126–Q130	5	__
TOTAL	**130**	__	

Revision Priority Matrix

Your Section Score	Priority
<50%	**RED** -- Revise this section immediately (re-read D1 chapter)
50–69%	**AMBER** -- Focused revision needed (review D6 quick reference + weak areas)
70–89%	**GREEN** -- Maintenance revision (review errors only)
90–100%	**BLUE** -- Exam-ready (move on to viva practice)

Compiled for the Weave Psychotherapy series. Companion to D1–QuestionBank-DeepStudy.md.

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